Evidence Submission

Fellowship Founders Pitch · Ideation Stage

Allan Gray Orbis Foundation Fellowship · E² Investments · 2025

⚠ **SAVE THIS DOCUMENT AS A PDF BEFORE SUBMITTING**

Step 1 — Complete all sections in Microsoft Word.
Step 2 — Delete all example content (blue blocks) before saving.
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Submissions in any format other than PDF will not be reviewed.

About the Ideation Stage — who this is for

The Ideation stage is the very first stage of the Fellowship Founders Pitch accelerator. It is for founders at the beginning — people who have identified a problem they believe is real and worth solving, and have started doing something about it.

You do not need a product. You do not need paying customers. You do not even need a fully formed business idea. What you need is: a genuine problem, some evidence that it is real beyond your own intuition, a direction for how you might address it, and the kind of self-directed action that signals you will follow through.

The Ideation programme develops you toward graduation criteria that include a validated problem statement, a working proof of concept, early market adoption evidence. You are not expected to have those things now. You are expected to have the raw material that coaching can develop into those things.

Who belongs here:
• You have identified a specific problem that real people experience
• You have spoken to or observed at least 3 people who have this problem
• You have some idea of how you might solve it even a rough direction
• You have done something on this idea without being asked or funded

Who does not belong here:
• You have an interesting topic area but no specific problem and no customer contact
• You have spoken to no one about the problem and have done nothing unprompted

Purpose of this document

This document is where you show the panel the evidence behind your application. Your written answers describe what you have observed and what you are thinking — this document proves you have done the work.

Section 1 is the hard gate section. Without documented evidence of real customer or problem conversations, your application will not advance.

Evidence at Ideation stage does not need to be formal. Handwritten notes from a conversation, a WhatsApp screenshot, a photo of a problem in the real world, or a sketch of an idea all count. What matters is that the evidence is real.

You can insert images as google drive links to save space but ensure that the links open for other people besides yourself.

Each section has a completed example in blue using StockAlert — a fictional stock-tracking tool for informal traders. Read each example, delete it, then complete the section with your own evidence.

Tip: honest evidence of a small, real observation is more compelling than a polished description of a problem you have not actually investigated.

CANDIDATE DETAILS

Name & Surname	Pearl Mothiba
Venture / idea name	SPM SMART HEALTH
Nature of business	Health Solutions
Date submitted	24 May 2026

1 Problem Validation Evidence ■ HARD GATE

Documented conversations or observations — minimum 3 required — hard gate

Instructions — Hard Gate

Document every conversation or observation you have conducted about the problem. You must have a minimum of 3.

Each block below represents one conversation or observation. Fill in as many as you have.

Accepted evidence: notes from a conversation, a WhatsApp message exchange, a voice note you took after speaking to someone, a photo of the problem in the real world, a video, or any other record showing you engaged with real people or real situations.

Your evidence does not need to be polished. It needs to be real.

Delete the example block (in blue) before submitting.

Example — Conversation 1 *(delete this block before submitting)*	
Name / role	*Auntie Gladys — informal trader, 14 years, selling fruits and vegetables in Johannesburg CBD.*
Date & method	*Saturday 12 October 2024 — in person at her stall, approximately 40 minutes.*
What I asked	*How do you currently track which stock is running low? What happens when you run out of something? How much do you think you lose when that happens?*
What they said	*She uses a paper notebook but rarely has time to check it. Runs out of mangoes and tomatoes most often — sometimes twice a week. Estimates she loses R300-R500 on a bad week when a customer walks away. Said she would pay for something that sends her a reminder.*
What I learned	*The problem is not awareness — she knows she runs out of things. The problem is timing. She needs a trigger at the right moment, not a tracking system she has to remember to check.*

Conversation 1	
Name / role	Dr [redacted] A junior doctor in the western cape
Date & method	21-22 April 2026- WhatsApp messages
What I asked	

	1. Are vital-sign collection or triage delays a contributor to patient long waiting times. 2. Could health ATMS reduce patient waiting time? 3. What are the bigger operational pain points need to be solved? 4. What happens within (1-2) hours of patient waiting to be triaged that they become worse? 5. When designing health ATMS's what would you worry most about?
What they said	1. Yes, though not a significant contributor in comparison to waiting time from triage to being seen by a health professional. In their institution an app called **hectis** is used to track patient waiting times from arrival to triage. On average it takes 1-2 hours for a patient to be triaged and up to 12 hours to be seen by the doctor after being triaged (it depends on how ill the patient is) 2. Triaging needs a human element. Vital signs along are not sufficient to decide a patient urgency or color code. It depends on the problem and how they appear as well. 3. More staffing, triage bays, bed are needed the most. 4. A critical pitfall in the current system occurs when patients wait too long and deteriorate. While critical "Red" patients are seen immediately and "Orange" cases within 10–20 minutes, most of the ED pool consists of "Yellow" (urgent) and "Green" (stable) patients. These patients are organized by color codes and sit unmonitored in waiting areas or outside. Because doctors spend hours treating highly complex, life-threatening cases, these waiting patients can wait long enough for their health to complicate, and some tragically die in the waiting area. If a patient isn't seen in their allocated time, there are theoretical rules to re-triage them and update their

	color code, but this is a major failure point in a crowded setting. 5.If patient vitals are only triage investigation being done without the human element, patients will be incorrectly color coded which can have catastrophic consequences e.g. if a red patient is coded lower and care is delayed.
What I learned	I learned that my initial assumption about building a faster initial triage machine (like a health kiosk) was solving the wrong problem, because automated vitals completely strip away the crucial clinical "human element" needed to safely categorize a patient. Instead, this conversation revealed the true, deadly systemic gap: the waiting room blind spot. Once the HECTIS system logs a patient's initial triage, they are sent to wait among the "Green" or "Yellow" pools for up to 12 hours. During this massive window, there is no continuous monitoring. If a patient's internal physiology deteriorates (shifting from a stable Green to a critical Orange), the system completely fails to notice.

Conversation 2	
Name / role	Dr [redacted] a GP at Rondebosch Riverside Mall
Date & method	22-April 2026. I requested a quick meeting on WhatsApp, and I conducted the interview in person at his practice.
What I asked	1. Which public hospital have you worked at? 2. Describe that hospitals triage process 3. What happens if a patients health condition worsens after triage?

	4. What are your thoughts on health ATMs specifically designed to perform triage?
What they said	1. Victoria Hospital 2. Patient's vitals readings are taken, and a physical and neurological observation is done by the nurse. There is an app used that guides the nurse through the triage process and the nurse inputs all the data collected. All the doctors have access to the data. Depending on the color code of the patient, patient waits to be seen by the doctor. 3. In the waiting area there aren't nurses, they are mostly occupied doing triage, so if a patient condition worsens sometimes a family member they came with will go and find help. There have been cases where patients die. At times some doctors take initiative to wait in the waiting area to observe the patients. 4. Provided the health ATM is accurate, it will help to reduce the pressure from nurses, and the ATM can have an additional feature where patients can press a button to signal their condition is getting worse
What I learned	If a patient does not have their family member waiting with them in the waiting area, it would be difficult for the patient to find assistance provided their condition worsens (hence the few deaths that occurred).

Conversation 3	
Name / role	Dr [redacted] a doctor at CA Cape Doctors Claremont

Date & method	28-April 2026- interview at the practice
What I asked	1 Explain Triage process 2 Are their cases where cases have been under- triaged and what is the cause? 3 What happens if a patient's health condition worsens e.g., they were green and after some time they become yellow? 4 What happens if a patient health condition worsens before triage?
What they said	1. When a patient is triaged, their symptoms are asked, vital readings are taken and general physical observations are done (eye bawling the patient). The process is complex and it differs case by case. For example, a patient was stabbed and, but their vital signs are normal, the nurse must then check where they were stabbed, if they are bleeding heavily or not. Sometimes patients exaggerate their pain levels usually patients who have chest pains or headaches so that they are prioritized this is the reason why vital signs are also taken. 2. There are some errors at times when a patient is under-triaged. The causes may be non-accurate vital signs taken. Sometimes a patient is triaged green but late their condition worsens. The vitals are taken at an instance; they are not continuously being triaged because it is very busy in the triage area. The only time a patient is triaged again is if their initial vital signs are not good 3. There should be nurses observing the waiting area; thus nurses can quickly spot the patients that become very ill.

	4. The nurses in the waiting area should spot such patients.
What I learned	I learned that triage is static rather than continuous. Vitals are only captured at a single instance because triage areas are too overwhelmed to re-assess waiting patients. This creates a dangerous vulnerability: a patient's initial vitals might mask a worsening condition, leading to under-triaging. Furthermore, hospitals rely on the assumption that waiting-room nurses will "spot" sinking patients; however, in understaffed public facilities, these nurses are entirely occupied with administrative triaging, leaving the waiting pool completely unmonitored.

Conversation 4

Name / role	17 UCT Students
Date & method	16 May 2026- A google form shared on UCT residential WhatsApp groups
What I asked	1. When was the last time you visited a hospital or clinic? 2. What time was it? 3. How did you get to the hospital 4. What best describes your visit? 5. What best describes your visit? 6. What was the first thing you noticed when you arrived? 7. How was the check-in/registration process? 8. Which hospital or clinic were you at? 9. How long did it take to be triaged (initial assessment with the nurse)? 10. How long did it take you to see the doctor after being triaged 11. While waiting, did staff continue checking on you or how you were feeling? 12. If you started feeling worse while waiting, did you feel there was a clear way to get help quickly? 13. Looking back now, what part of the experience affected you the most?

What they said	Response
What I learned	From the responses, students who visited private hospitals mostly had a good overall experience compared to student who visited public hospitals. The responders who visited public hospitals stated that no nurse checked up on them or asked them how they were feeling, they were in the waiting room for long hours with no clear pathway of asking for help provided their condition become worse. They felt isolated, anxious, uncertain and mistreated. One response stated that one person died in the waiting area. Another respondent stated they were placed in a secluded area with barely any medical staff walking around. One respondent had to wait for 8 hours by themselves with no one checking up on them.

Conversation 5	
Name / role	[redacted] health care worker(nurse)
Date & method	12-May 2026, Instagram DM
What I asked	1. How does triage workflow work 2. What delays are in the triage workflow 3. What works well in the triage workflow 4. In your ideal world, how does patient workflow look like from the time patient enters the emergency unit
What they said	1. 1st contact in an emergency center is triage. They categories patients based on the severity of their medical conditions or presenting symptoms. The life-threatening person will be seen first followed by orange and green. The dead will be put aside awaiting mortuary 2. Patients fight to be seen first especially green/stable patients and that slows down the flow of patients. Few staff members working. Sometimes staff members must perform CPR in triage that completely stops the flow of patients. Sometimes staff must accompany patients inside due to being unstable.

	3. Vital sign equipment. Well trained staff. Basic vitals they look for to categorize a patient are easy to get from patients 4. Section 1 with a nurse and a doctor that triage patients. In the same section, another nurse and doctor are waiting to take over and accompany life threatening class of patients. 2nd section is a triage and treatment for green patients. 3rd section is medical and trauma section with each side having their own staff, plus radiographers to operate imaging such as X-Ray on site plus parking for ambulance and pharmacy next doctor.
What I learned	Medical staff must split attention from triaging patients, to accompanying unstable patients inside, to observing patients who have been triaged.
Observation 1	
Name / role	Pearl Mothiba, at Groote Schuur triage area.
Date & method	30 April 2026 12h00 -12h30
What I asked	I asked the security guard if I can sit and observe the triage area. The security guard refereed me the triage nurse and the triage nurse referred me to the triage doctor. The triage doctor said I NEED a letter of approval, I can't just sit and observe due to hospital rules and patient confidentially.
What I observed	I waited approximately 30 minutes for the triage doctor's response. While I was waiting, I observed the following: patients who came by ambulance and could not walk were triaged first. Patients who came by ambulance and could walk had to wait to be triaged. Other patients came in, sat down, the security gave them form to fill in. There were about 6 patients waiting to be triaged. Out of the 6, I patient was in the process of being triaged, the nurse took some vitals filled them in on the system, then after some time, this patient was moved to another waiting area. There was an elderly man sitting next to me with an oxygen mask on. His eyes were closed, he was holding his chest, I could tell he was struggling to breathe. When I left the man had not been triaged. Across me there was a woman wearing a pink gown, her eyes were closed, she looked like she was in a lot of pain, she was holding a form in her hand. I found her there

	when I arrived and when I left, she had still not been triaged. Out of the 6 patients only 1 patient was with a family member. In the waiting area there were 2 nurses triaging the patients, the doctor was in another area.
What I learned	Patients must wait for some time before they can be triaged. Most patients come to the hospital alone and if the nurses are not checking the patients they could die e.g. the man sitting next to me was very weak(the paramedics helped him walk to the waiting area), if his health condition became worse he would not have the strength to stand up and look for help. The nurses are working under a lot of pressure, once they triage a patient, this patient moves to another area, and they move on to the next patient.

Article 1	
Name / role	Woman breaks silence after Victoria Hospital Tragedy
Date & method	Published- 02 April 2026.
What article was about	Grede Kruger explained that her life partner Philip Kruger experienced intense pain on 4 March 2026 on his calf and numbness on his foot. Grede took Philip to Victoria hospital, he registered at reception and directed to the triage area. They waited for some time then Grede knocked on the triage room, the nurse then took Philip's vitals, and he was told he will only be able to see the doctor after 8 hours. While waiting Philips pain worsened, Grede decided to take him home. They returned to the hospital later that morning and a general practitioner examined Philip and transferred him to Groote Schuur hospital. Unfortunately, Philip has a massive stroke, went into a coma shortly after being admitted at Groote Schuur hospital. He passed away early morning of 6 March 2026
What I learned	The nurses triaged Philip and told him he had to wait 8 hours. While Philip was waiting, they did not monitor his condition, they didn't recheck his vitals, or the condition of his leg, they didn't check on him when he said his pain intensity increased (hence Grede left with Philip). If Philip was alone, he would not have been able to stand up (his one leg was in intense pain) and look for help.

Article 12	
Name / role	Patient dies at Helen Josephs waiting room after being ignored for 24 hours
Date & method	27 July 2021
What article was about	Dube arrived at the hospital at 1pm 26 July 2021 with her mother Nqobile. At 9pm Nqobile was told to leave the hospital and that her child will be admitted. Nqobile returned to the hospital following day at 5pm, she found her child in the wheelchair, in same position as she left her the previous day and screamed for help from the nurse, but the nurse told her told that she is very busy. Her daughter had passed away.
What I learned	The nurses did not monitor Dube's condition until she passed away, the nurses were not even aware that Dube passed until her mom came back the following day.

What is the single most important thing you learned from these conversations or observations?

The insight that most changed or confirmed your thinking about the problem.

Example: *The problem is not that traders do not know they run out of stock — they know. The problem is they do not get a signal at the right time to act on it. Any solution that requires them to remember to check something will fail. It needs to push information to them.*

Initially, I believed the core issue was the speed of the initial triage process and that we needed a faster intake mechanism (like a health ATM) to relieve pressure on nurses. However, my primary interviews, community survey, and real-world observations completely shifted my focus. The true, life-threatening flaw in the system is that triage is treated as a single, static event rather than a continuous process. Once a patient is triaged and assigned a color code, they are sent to a waiting area where they sit completely unmonitored for hours (up to 12 hours). Because staff are overwhelmed and trapped in the intake cycle, they do not check on patients. If a patient's condition deteriorates in the waiting room, they have no active pathway to signal for help. They must either rely on a family member to frantically hunt for a nurse, or—as documented in the Victoria and Helen Joseph hospital cases—they tragically deteriorate and die silently in their chairs. Therefore, the solution must pivot from *faster initial triage* to continuous, automated patient monitoring and distress signaling in the waiting ecosystem.

Insert any supporting evidence below — photos, screenshots, voice note transcripts, or other records.

Example: photo of a trader's paper notebook, WhatsApp conversation about the problem, photo of an empty stall shelf.

Google drive link

The links to all the articles:
Woman breaks silence after Victoria Hospital Tragedy
Patient dies at Helen Josephs waiting room after being 'ignored for 24 hours'

2 Problem Statement

Your clearest articulation of the problem based on what you found

Instructions

Now that you have documented your evidence, write your clearest statement of the problem. This should be grounded in what you actually found — not what you assumed before you started.

A strong problem statement identifies a specific person, a specific situation, and a specific cost. It does not describe a broad social issue — it describes a real experience that a real person has.

Who has this problem?

Describe a specific person or group — not a broad demographic. Who are they and what is their situation?

Example: *Informal traders who operate in high-density urban markets (Johannesburg CBD, Soweto, Khayelitsha) — primarily women, running fruit, vegetable, or mixed goods stalls, managing stock across 2-5 suppliers without a smartphone-based system.*

Patients who visit public hospitals in Cape Town. These patients are ill and they want their health condition to be treated.

What exactly is the problem they experience?

Describe the specific moment or situation where the problem occurs. What happens and why is it a problem?

Example: *They run out of high-margin stock (mangoes, tomatoes, onions) at unpredictable times and do not realise until customers leave or ask for something they cannot supply. By the time they notice, the supplier visit has passed or the market is busy and they cannot leave to restock. They lose the sale and the customer sometimes does not return.*

After undergoing their initial, single-instance triage assessment, these patients are left completely unmonitored in overcrowded waiting areas for anywhere from 2 to 12 hours. Because ED staff are severely constrained and entirely occupied with conducting new intakes or treating critical "Red/Orange" cases, there is zero routine re-triage or vital-sign monitoring of the waiting pool. If a patient's physiological condition suddenly worsens (e.g., transitioning from a stable 'Green' to a critical 'Orange'), there is no built-in mechanism to detect this change. The patient is trapped in a systemic blind spot with no clear, accessible way to quickly signal clinical staff for emergency assistance before their condition becomes irreversible.

What does this problem cost them?

Time, money, opportunity, safety, dignity — be as specific as you can.

Example: *Estimated R300-R600 per week in missed sales based on conversations with 8 traders. Also costs them in customer trust — a customer who cannot find what they want twice is likely to find a different trader. The biggest traders I spoke to estimate they lose 1-2 loyal customers per month to this.*

Loss of Life & Fatal Medical Complications: In severe instances, this systemic gap costs patients their lives. As documented in public records and interviews, patients experience massive strokes, organ failure, or silent cardiac arrest, passing away directly in waiting room chairs or wheelchairs without staff even realizing it. **Severe Psychological Distress:** For surviving patients, the experience inflicts immense trauma. My survey of 17 respondents highlighted profound anxiety, helplessness, and acute confusion caused by a total lack of communication and the fear of dying unnoticed. **Prolonged Physical Agony:** Patients endure hours of escalating, unmanaged pain without any clinical intervention or re-evaluation

3 Solution Thinking

Your current best idea for solving the problem

Instructions

Describe your proposed solution and any proof of concept you have developed. At Ideation stage you are not expected to have a working product — a sketch, a spreadsheet, a paper prototype, or a mockup all count.

Be honest about what exists and what you have learned from it. A tested rough prototype that revealed a flaw is more impressive than a polished design that was never tested.

What is your proposed solution and how does it work?

Describe what it does and what form it takes. Why does this approach solve the problem better than what exists?

Example: *A WhatsApp-based stock alert system. Traders send a message when they notice stock is running low (e.g. 'mangoes 5 left'). The system logs it and sends a reminder at a set time before their supplier visit. Works on any phone with WhatsApp — no app download, no smartphone required. Traders I spoke to already use WhatsApp constantly so friction is low.*

A smart adjustable wristband. After patient is triaged, the nurse gives them an adjustable wristband to wear (the wristbands are numbered, the same number as the patients in the waiting area). Connected to this wristband is a comfortable clip that goes on their finger. This finger clip continuously tracks their heart rate and oxygen levels. The wristband automatically sends the patient's vitals to the central computer screen at the nurse's desk or the HECTICS app (data transfer is completely offline using wireless ESP-NOW). If a patient's numbers drop to a dangerous level, the wristband's microcontroller instantly broadcasts an emergency trigger to a network of heavy-duty wireless buzzer pods mounted to the walls or ceilings throughout the emergency waiting zone. Rather than triggering a generic alarm sound, these distributed buzzer pods blast a distinct acoustic sequence that chimes out the exact number assigned to that specific patient's wristband (e.g., fourteen distinct buzzer pulses for Wristband #14), allowing mobile clinical staff to instantly identify the patient.

This solution considers that healthcare workers are constantly moving and must be able to hear a critical alert; it considers that hospitals are intensely noisy environments where standard machine alarms easily blend together, overcoming this by introducing a highly unique, numeric-pulsed sound; and it directly considers the infrastructural reality that many public or deep rural facilities completely lack reliable Wi-Fi connections, cellular data, or electricity backups, resulting in a cheap, fully reusable solution.

Have you built, sketched, or prototyped anything? What exists right now?

Even a paper sketch, a spreadsheet, a Figma mockup, or a group you set up to test an idea counts. Describe what you made and what you learned.

Example: *Built a manual version using Google Sheets and a WhatsApp group. 3 traders (my aunt and 2 of her neighbours) used it for 4 weeks. They logged stock via WhatsApp and I manually sent reminders. Main finding: they used it consistently for the first 2 weeks then stopped. Asked why — they said the reminders came too early. Adjusted timing and they re-engaged. This taught me the alert timing is the key variable, not the tracking itself.*

I have not built anything yet; I am still designing the solution.

Insert a photo or image of your prototype, sketch, or mockup below.

Example: photo of a paper prototype, Google Sheets screenshot, WhatsApp group screenshot, Figma wireframe.

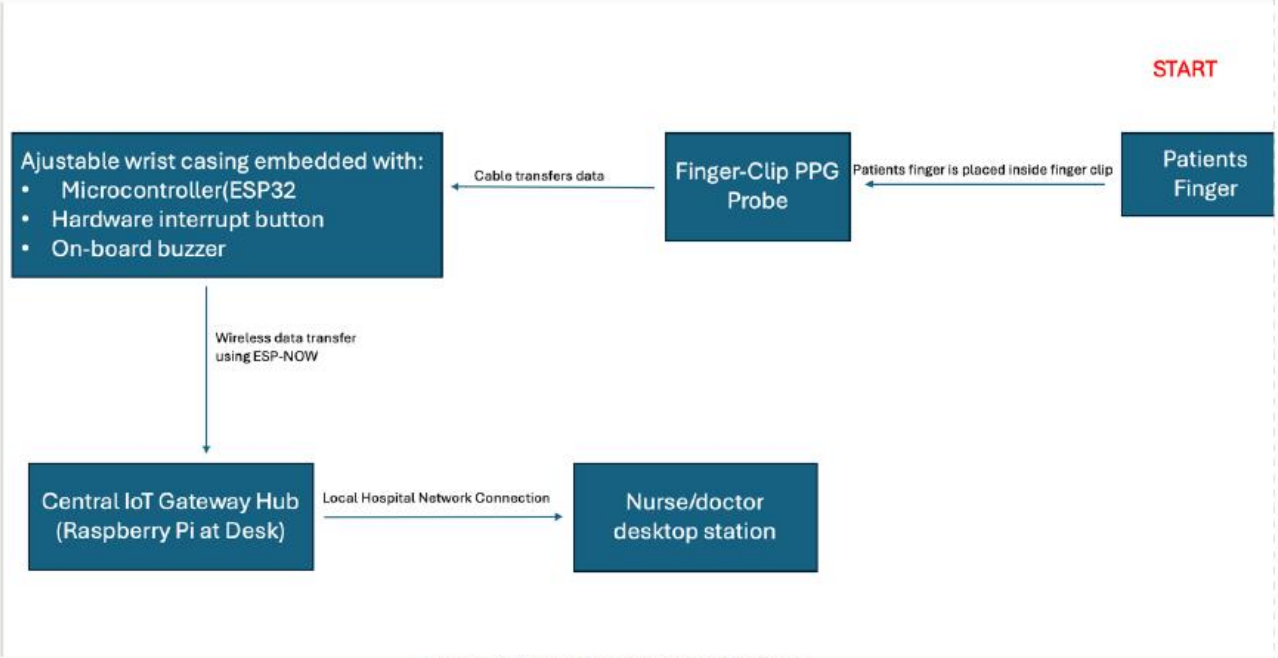
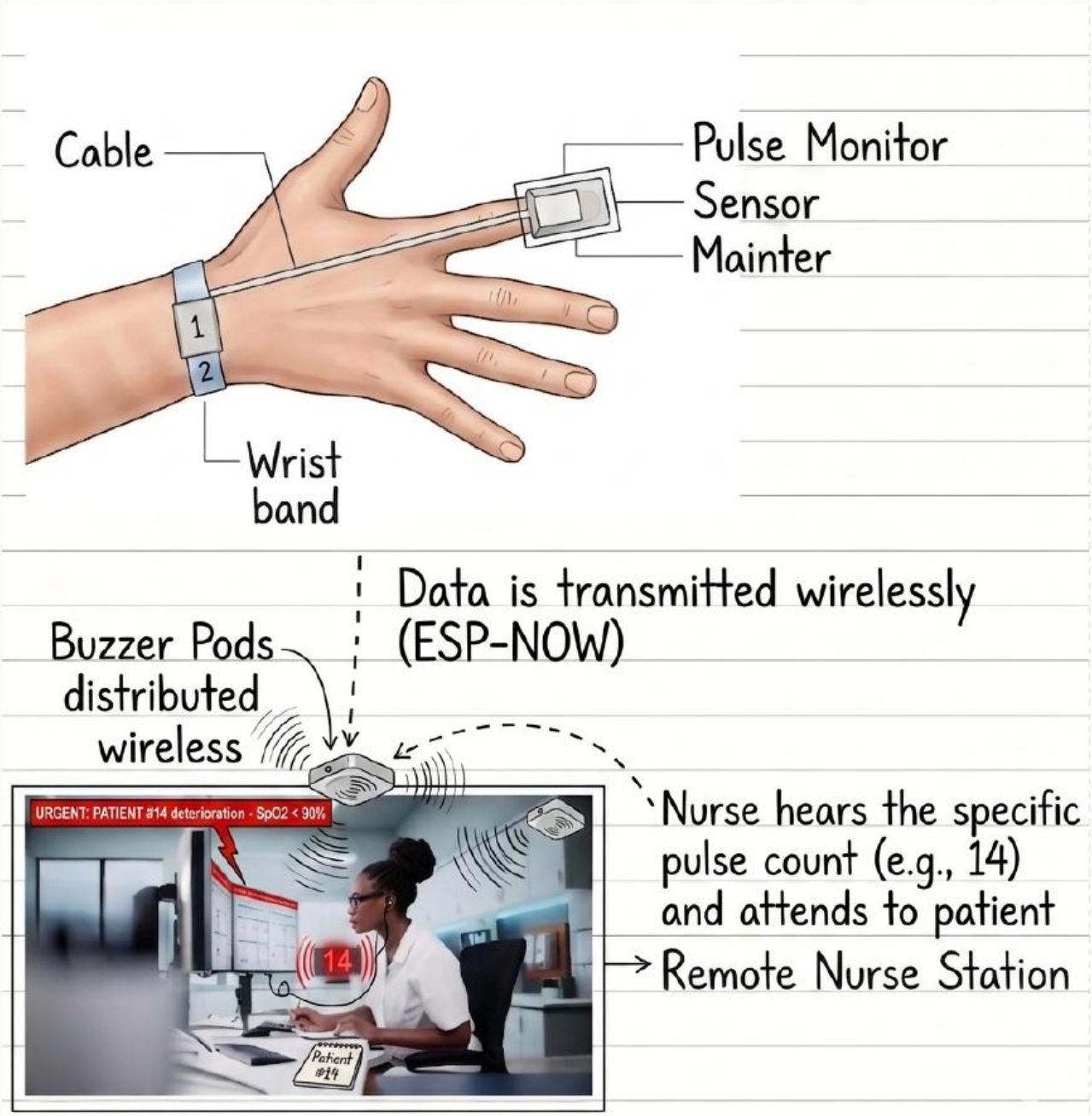


Figure 1:System Integration diagram



4 Self-Directed Action

Everything you have done on this idea without being asked or funded

Instructions

List every concrete action you have taken on this idea before submitting this application. Be specific — what did you do, when, and what happened as a result.

This section is about demonstrating initiative. The panel is looking for evidence that you are the kind of founder who acts without being prompted. One specific action you actually took is worth more than ten things you plan to do.

List every action you have taken on this idea without being asked.

Built something, researched something, spoke to someone, tested something, spent your own money or time — anything unprompted.

Example: *Spent 3 weekends in the CBD observing and speaking to traders (October 2024). Built a Google Sheets tracker and ran a 4-week manual test with 3 traders. Interviewed 8 additional traders to validate findings. Mapped 14 potential customers within 3 streets of the CBD. Started a WhatsApp group with 6 traders to share product ideas. Read two City of Johannesburg reports on the informal economy. Sketched 3 different versions of the solution based on feedback.*

I spend hours waiting for medical doctors to finish their consultation so that I can conduct interviews. I went to Groote Schuur hospital to observe the triage area before getting kicked out. I talk to any medical professional anywhere I get a chance. I ask acquittances to connect me with their family members that are medical professionals. I have sent medical doctors DMs on Instagram some respond most do not respond. I speak to some medical students from Med-school and I ask them questions.

What is the hardest thing you have faced working on this idea — and what did you do?

A real obstacle and a real response. If you have not faced one yet, describe the biggest uncertainty you are sitting with.

Example: *The traders stopped using the manual test after 2 weeks. My first instinct was that the idea was wrong. Instead I went back and asked why. Turned out the timing was the issue, not the concept. Adjusting the reminder time re-engaged them. This was the moment I realised that the idea has legs — but only if I get the details right.*

It is difficult to interview medical doctors more difficult to interview nurses. I had to go to the practices and wait for the doctor to finish all the consultations, then conduct a short interview. I had to do this when interviewing 2/3 doctors. Nurses are mostly found in hospitals, they are mostly inaccessible because I need permission from the hospital to interview any of their staff members, hence I sent an Instagram message to a nurse. I went to Groote Schuur thinking I can just observe the triage area, but I told I had to leave and I can only do so with hospital permission. I tried conducting my lecturer that works on bio-medical projects because he has connected at medical school, but he could not help me

out. I resorted to sending google form to different student residential groups. I am also predicting that it is going to be difficult to test such a solution, hence I have started connecting with entrepreneur doctors on LinkedIn, learning from entrepreneurs from UCT MedTech on how their devices are approved to market.

5 Market & Value

Who you are building for and why this matters at scale

Describe your target customer in one specific profile.

Not a demographic category — a real person. Who are they, what is their daily life like, and why does your solution matter to them?

Example: *Gladys, 47, has run a fruit and vegetable stall in the Johannesburg CBD for 14 years. Wakes at 4am to buy stock from the market before her stall opens at 7am. Runs the stall alone Monday to Saturday. Has no accounting software, uses a paper notebook she barely has time to check. Makes R800-R1,200 on a good day. Her biggest fear is losing her regulars to the stall next door.*

Sipho, 30, is a construction laborer and the only breadwinner at home. He suddenly develops an aggressive, high-grade fever and severe breathing complications while at his home in Cape Town. He is rushed to Groote Schuur Hospital via ambulance at 18:00, arriving entirely alone because his family lives in another province. After a 30-minute wait, the overwhelmed intake nurse takes his initial vitals, registers him on the HECTIS system, and color-codes him as **Yellow (Very Urgent)**. He is directed to an overcrowded, chaotic waiting hall and told it will take several hours before a doctor can examine him. He feels completely helpless, isolated, and highly anxious. Because the emergency department is severely understaffed, the triage nurses are completely trapped at the intake desk performing new assessments; no medical personnel are stationed to monitor or check on the patients sitting in the waiting area. Sipho is left entirely on his own with no one tracking how he is feeling. Within 45 minutes of sitting down, his internal physiology begins to rapidly deteriorate: his heart rate spikes and his oxygen levels plunge. Because he is entirely alone and physically incapacitated by severe shivering and weakness, he does not possess the strength to stand up, navigate the crowd, or look for help. Without my solution, Sipho is trapped in a systemic blind spot—he will slide quietly into septic shock, collapse out of his chair, and suffer an irreversible stroke that leaves him permanently paralyzed on his left side. Not only is Sipho paralyzed, but his family also no longer has a provided. With the SPM Smart wrist band, the nurses could attend Sipho before he got into septic shock.

How large is this problem? How many people have it?

Your best estimate with your reasoning. You do not need a formal market size calculation.

Example: *Johannesburg CBD alone has over 8,000 registered informal traders. The City of Cape Town estimates 150,000 informal traders nationally. From my conversations, the stockout problem is consistent across different types of traders and different areas. Even at 1% penetration nationally that is 1,500 potential customers.*

All public hospitals in South Africa- 420 hospitals. All public clinics- 3500. Over 80% of South Africa population (~50 million people) rely solely on public health care system for survival.

6 Additional Evidence

Optional — anything else the panel should see

Instructions

Use this section for any evidence that strengthens your application but does not fit the sections above.

Examples: a news article showing the problem is real and growing, a research paper, a photo of the problem context, a screenshot of a community conversation about the problem, or a message from someone you spoke to.

Note: Optional. Do not pad it.

[Insert additional evidence — optional]
Insert → Pictures in Word

Additional context or notes

PRE-SUBMISSION CHECKLIST

Before saving as PDF, confirm:

☐ I have deleted all example blocks (blue sections) from the document.

☐ Section 1 has a minimum of 3 completed conversation or observation blocks.

☐ Section 1 includes at least one photo, screenshot, or other real-world evidence record.

☐ Section 2 problem statement is grounded in what I actually found — not what I assumed before I started.

☐ Section 3 describes my solution clearly and mentions what I have actually built or sketched.

☐ Section 4 lists specific concrete actions I have taken — not plans.

☐ Section 5 describes a real customer profile based on someone I have actually spoken to or observed.

☐ My candidate details at the top are filled in correctly.

☐ I have saved this file as a PDF: File → Save As → PDF.

☐ My file is named: FirstnameLastname.pdf

☐ I am uploading the PDF — not this Word file — to the Microsoft Form.

⚠ **Final reminder — have you saved this as a PDF?**

File → Save As → change file type to PDF → name it FirstnameLastname.pdf → Save

Only PDF submissions will be reviewed. Do not upload this Word file.

Applications without at least 3 documented conversations or observations in Section 1 will not clear the hard gate.